

Run of show

25 MINUTES

Post this by the workstation at both sites. The facilitator calls the time out loud at 0:07, 0:16 and 0:22.

START	MIN	SEGMENT	WHAT HAPPENS	DRIVER
0:00	1	Launch	Roles named, one objective stated.	Facilitator
0:01	2	The hook	One-liner + one provocative datum. Then stop.	Presenter
0:03	4	History & exam	One screen. No labs, no imaging yet.	Presenter → Scribe
0:07	4	First pass	Problem representation, 3–4 diagnoses, can't-miss.	PGY-1
0:11	5	Second pass	Framework, pruning, the discriminating test.	Senior
0:16	3	Labs & imaging	Scribe strikes refuted items in red.	Presenter → Scribe
0:19	3	Converge	Plan stated. Faculty enters here.	All + Faculty
0:22	2	Reveal	Diagnosis, what was missed, and why.	Presenter
0:24	1	Close	2–3 big-picture points. Board exported.	Facilitator

GROUND RULES

- Cameras on. All of them.
- Names, every time — when calling on someone and when crediting an idea.
- Nobody rescues, corrects or finishes the intern's sentence during the first pass.
- "I don't know" gets said by the facilitator and faculty first.
- Hug the case. Tangents go back to this patient.
- No new content in the last three minutes.

THE ONE RULE THAT MAKES THE REST WORK

History and exam come before the first pass — the intern cannot build a problem representation without them. What gets withheld is the diagnostic data: labs, imaging and micro wait until after the intern has committed. That withholding is what makes the commitment worth anything.

TODAY'S ASSIGNMENTS

Date		Site	
Presenter		Whiteboard scribe	
PGY-1 discussant		Senior discussant	
Faculty		Facilitator	

Objective for today: _____

Case Presenter

0:01 — 2 MIN

You own the data and the pace of the reveal. The hardest part of this job is not answering.

BEFORE REPORT

- Pick a case that is relevant, realistic and instructional. Vary common versus rare.
- Master it — read the chart end to end, write down every question it raised, then read to answer them.
- De-identify completely. No MRN, no dates of service, no burned-in identifiers on images.
- Build no more than 8 slides on the template. 6 is better.
- Send the one-liner to both discussants 24 hours ahead. Send the objective to the scribe.
- Test the screen-share handoff once before the day of.

DURING REPORT

- Open with the hook, not the demographics — put the group at the most provocative moment of the story.
- Release data in the two planned blocks only. History and exam first; labs and imaging after the intern commits.
- Answer the questions asked. Volunteer nothing.
- Keep a poker face. No tone or pacing tells when someone lands on it.
- Hand the share to the scribe at each pause, on the facilitator's cue.

THE HOOK, SAID OUT LOUD

“[Age] [sex] with [relevant background] presents with [chief complaint] for [duration]. [One provocative datum — the vital sign, the exam finding, or the image that made this case worth 25 minutes.]” — then stop talking.

DOES NOT

Narrate the whole case front to back. Correct the discussants. Reveal the diagnosis early because the room went quiet.

AFTER

Post the slides and the exported board to the residency channel within 24 hours.

YOUR BLOCK IN THE 25 MINUTES

	Hook	Hx + exam	First pass	Second pass	Labs	Converge	Reveal	
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0:00

0:25

NOTES

Whiteboard Scribe

ALL 25 MIN

You own the board. It is the group's thinking made visible — so write their words, not yours.

BEFORE REPORT

- Open the Zoom Whiteboard 10 minutes early and lay in the zones below.
- Fill the header: date, de-identified case ID, the objective.
- Confirm co-annotation is on. Post the whiteboard link in chat for second-monitor viewing.

CASE ID | DATE | OBJECTIVE: _____

PROBLEM REPRESENTATION (PGY-1, one sentence, out loud)
____-year-old with _____ presenting with _____ over _____

KEY DATA (black)
Hx · exposures
· birth, immunizations
Exam · vitals, growth
· pertinent pos / neg
Labs + imaging · block 2

DIFFERENTIAL (blue → red)
Framework: _____
1. _____
2. _____
3. _____
Struck items stay visible.

DISCRIMINATORS (green)
Next test: _____
If pos → _____
If neg → _____
Pending: _____
Bias watch: _____

THE COLOUR CODE

- Black** Objective case data, as the presenter releases it
- Blue** Live differential — anything said aloud by anyone
- Red** Struck through when refuted, with the refuting feature written beside it
- Green** Plan, pending data, discriminating tests
- Amber** Teaching points and anything faculty adds after 0:19

THREE HABITS

Never erase — a crossed-off diagnosis with the reason beside it is the most useful thing on the board. Write it the way they said it; if the phrasing is vague, ask them to sharpen it rather than sharpening it yourself. Draw a line across the board when block 2 lands — everything above it was reasoning without those results.

DOES NOT

Add items nobody said. Tidy up sloppy reasoning into clean reasoning. Argue the case — the scribe is not a discussant.

Export the board as a PNG at 0:24 and post it before leaving the call. The board is the artifact of the session, not the slides.

YOUR BLOCK IN THE 25 MINUTES

	Hook	Hx + exam	First pass	Second pass	Labs	Converge	Reveal	
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0:000:25

SCRATCH — DRAFT THE ZONES HERE BEFORE YOU OPEN THE BOARD

PGY-1 Discussant

0:07 — 4 MIN

The first pass. Commit to a short list — a defensible four beats an unfalsifiable twelve.

BEFORE REPORT

- You get the one-liner only, 24 hours ahead. No chart access, no labs.
- Review your own approach to that chief complaint — not the diagnosis.
- Come ready to think out loud, not to be right.

YOUR FOUR DELIVERABLES, IN ORDER

1. Problem representation — one sentence with semantic qualifiers: age, relevant epidemiology, tempo, key features.
2. Three to four diagnoses, each with the reason it is on the list.
3. The can't-miss, named as such, even if it is unlikely.
4. The one piece of data you most want, and what each result would mean.

WHAT A STRONG FIRST PASS SOUNDS LIKE

“Three-year-old, previously healthy, two days of fever and refusal to bear weight, holding the hip flexed and externally rotated. I think this is septic arthritis of the hip until proven otherwise — the positioning plus the CRP is what worries me. Transient synovitis is my main competitor and I can’t separate them on exam. Osteomyelitis is possible. My can’t-miss is septic arthritis because of the joint-destruction clock. What I most want is the hip ultrasound.”

WHAT A WEAK FIRST PASS SOUNDS LIKE

“This could be infectious, inflammatory, oncologic or traumatic — so maybe leukemia, maybe JIA, maybe a bone tumor, maybe septic arthritis, maybe transient synovitis.” Five categories, no leader, no test. Nothing the board can act on, and nothing that can ever be crossed off.

DOES NOT

List every diagnosis you can think of. Breadth is avoidance, not thoroughness. Saying “I don’t know” out loud is expected and is never penalised.

Nobody interrupts, corrects or rescues you during these four minutes. That is the deal.

YOUR BLOCK IN THE 25 MINUTES

	Hook	Hx + exam	First pass	Second pass	Labs	Converge	Reveal	
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0:00

0:25

PREP — WRITE YOUR FOUR DELIVERABLES HERE BEFORE REPORT

Senior Discussant

0:11 — 5 MIN

The second pass. You are not producing a better list — you are turning the intern's list into an argument.

BEFORE REPORT

- You get the one-liner, plus the intern's differential if the facilitator shares it.
- Bring one organising framework to offer — anatomic level, anatomic layer, physiologic, pre/intra/post, mechanism, age, tempo.
- Identify one teaching artifact to reference: a table, an algorithm, a criteria set.

YOUR FIVE MOVES, IN ORDER

1. Organise into a framework

“Let me take this by layer, because that’s what the imaging will sort out. Skin and soft tissue — no cellulitis, off. Joint — septic arthritis and transient synovitis, both live. Bone — proximal femoral osteomyelitis, live. Referred — off; he localises.”
2. Prune out loud, with the reason

“Febrile at 38.6, CRP 8.4, refusing to bear weight — three of four Kocher criteria, past the point where I keep trying to talk myself out of it.”
3. Apply the illness script

“Classic transient synovitis follows a viral illness, the child is afebrile or low-grade, and they’ll usually weight-bear if you coax them. That is not this kid.”
4. Name the discriminating test

“Hip ultrasound, with aspiration if there’s an effusion. Effusion plus that CRP and I’m calling ortho tonight. No effusion pushes me to MRI, not back to synovitis.”
5. State confidence, trigger and bias

“Septic arthritis, about 70%. What would change my mind is a dry tap with a normal-looking joint — then I’m in bone, not joint. I’m watching myself for premature closure: I anchored on hip the second I heard the positioning, and nobody has looked at the smear.”

DOES NOT

Skip straight to the answer. Treat the intern's list as a first draft to be discarded rather than organised. Every move above is a claim the group can push back on — that is the point.

Ask the intern the question rather than correcting them: “You said hepatitis. Which one, and what would you send?”

YOUR BLOCK IN THE 25 MINUTES

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0:00

0:25

PREP — THE FRAMEWORK YOU PLAN TO OFFER

Faculty

ENTER AT 0:19

An expert on call, not the answer key. Your presence signals that report matters; your timing decides whether it helps.

BEFORE REPORT

- You will get a short priming note: the objective, when you enter, and what kind of contribution helps.
- Skim the topic so your contribution is depth, not recall.
- Tell the facilitator if a subspecialist, pharmacist or microbiologist should also be invited.

DURING REPORT

- Stay off the discussion until 0:19 — unless a patient-safety error is being taught as correct.
- Add what the residents cannot: pathophysiology depth, the evidence behind a guideline, the caveat from real practice.
- Answer what is asked. Redirect questions aimed at learners back to the group.
- One pearl, not a lecture. Sixty seconds, one point.
- Close with a two-sentence synthesis tied to the stated objective.

WHY THE CLOCK MATTERS

Well-meaning attending input, delivered early, ends the discussion it was meant to enrich. The six minutes you hold back are what allow an intern to be visibly wrong and a senior to correct course in public. At 0:19 the room has already done the work, and your contribution lands on prepared ground rather than replacing the preparation.

DOES NOT

Answer a question the facilitator posed to a resident. Correct a learner in a way that ends participation. Take over the case.

ACROSS THE YEAR

Expect the intern's block to get more demanding as the year goes on — by spring they are naming a framework, pruning explicitly and stating what would change their mind. As that happens the senior's block shrinks from five minutes to three. If you find yourself with less to add in June than in October, the format is working.

YOUR BLOCK IN THE 25 MINUTES

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0:000:25

THE ONE PEARL YOU PLAN TO ADD

Facilitator

ALL 25 MIN

Chief resident or designated faculty. Not a discussant — a conductor. You run the clock and protect the climate.

BEFORE REPORT

- Draw the two discussants the week before and post the assignment. Drawn names come off the wheel until everyone has had a turn.
- Send the one-liner to both discussants 24 hours ahead — and, when escalating the intern's ask, add one line naming the new demand.
- Send faculty their priming note.
- Confirm Zoom settings: whiteboard on, co-annotation on, multiple-share off, recording off.

DURING REPORT — WHAT YOU ACTUALLY DO

- Call the time out loud at 0:07, 0:16 and 0:22. Those three handoffs are where 25 minutes becomes 40.
- Call on people by name.
- Manage the spotlight and the share handoff between presenter and scribe.
- Read chat aloud so quiet contributions do not vanish.
- Redirect questions back to the group rather than answering them.
- Cut tangents: hug the case.
- Protect the first pass. Nobody speaks over the intern for four minutes.

THREE LINES WORTH HAVING READY

“Hold that — let’s let [name] finish the first pass.” “Good question. What does the group think before we ask Dr. [name]?”
“We’re at 0:16 — labs and imaging now, please.”

ESCALATING THE INTERN ACROSS THE YEAR

- Jul – Sep
- The four deliverables only. Senior block 5 min.
- Oct – Dec
- + the test that separates their top two. Senior block 5 min.
- Jan – Mar
- + the framework, named before the list. Senior block 4 min.
- Apr – Jun
- + explicit pruning and the trigger. Senior block 3 min.

THE ONE SIGN IT IS GOING WRONG

The intern's list gets longer after you escalate. That is covering, not committing — reaching for breadth to avoid being pinned to a choice. Back off one rung and say plainly that four with reasons beats eight without.

Escalate in the pre-report email, never on camera. Ambushing someone in front of the group costs you the safe-to-be-wrong contract.

YOUR BLOCK IN THE 25 MINUTES

	Hook	Hx + exam	First pass	Second pass	Labs	Converge	Reveal	
0:00								0:25

NOTES